Acute Chest Syndrome *Prodigy Anesthesia*

Acute chest syndrome is the single greatest threat to sickle cell patients and has a mortality rate of up to 20%. It is diagnosed by the development of new infiltrates on chest film and may be caused by atelectasis, pulmonary microembolic episodes, or pulmonary infection.

Acute chest syndrome is a potential complication of sickle cell disease. Symptoms include chest pain, cough, wheezing, and/or hypoxemia. When it develops, it is typically 2-3 days following surgery and requires aggressive hydration, oxygenation, transfusions to treat anemia, antibiotics to treat pulmonary infection, and occasionally nitric oxide may be used to treat pulmonary hypertension. The incidence of acute chest syndrome following abdominal surgery is 10-20%.

Barash PG, Cullen BF, Stoelting RK, Cahalan MK, Stock MC, Ortega R. Clinical Anesthesia. 7th ed. Philadelphia, PA: Lippincott Williams and Wilkins; 2013: 631-632.

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